


Gary L. Carrigan M 74 y.o. 6/19/1950 (MRN: 001256478)

999-999-9999 (H) PCP: Corales, Danilo, MD

General Info

Referral ID:	118137248	Received on:	5/6/2025
Class:	Outgoing	Type:	Consultation
Priority:	Routine		

Referring Info

Referred By

Provider:	Sekkath Veedu, Janeesh, MD
Location:	CHANDLER MARKEY

Procedures

REF361 - AMB REFERRAL TO MOLECULAR TUMOR BOARD

Diagnoses

C34.90,C79.51 (ICD-10-CM) - NSCLC metastatic to bone (CMS/HCC)

Notes

Provider Comments

5/6/2025 12:00 PM EDT by Sekkath Veedu, Janeesh

Case Submission Form: Molecular Tumor Board

Please complete the case submission and submit the Genomic Report to the Molecular Tumor Board Manager at mtb@uky.edu or fax to (859) 257-0100.

Physician Information

Requested timeframe for presentation at MTB

How would you like to receive recommendation letter? Email or Fax

Patient Demographics

Initials:

MRN: 001256478

Date of birth: 6/19/1950

Gender: male

Race: @RACE@ Ethnicity:

Primary Site of Disease: Histology:

Stage: No matching staging information was found for the patient.

Performance status:

Hepatic function:

Lab Results			
Component	Value	Date	Lab Results
BILITOT	0.3	04/22/2025	
Component	Value	Date	
ALT	19	04/22/2025	

Renal function:

Lab Results			
Component	Value	Date	Lab Results
BUN	17	04/22/2025	
Component	Value	Date	
CREATININE	0.45 (L)	04/22/2025	

Comorbidities:

How far willing to travel for clinical trial:

Zip code of home address:

Prior Therapies (Most recent first)

Current Therapy (if any)

Location of Measurable Disease (if any)

@TREATMENTPLANS@

@ONCHX@

 Triage Information

Decision: (none)

Priority: Routine

Schedule by date: 6/5/2025

 Progress Notes (Notes from 04/22/25 through 05/06/25)

Notes from 04/22/25 through 05/06/25

Progress Notes by Sekkath Veedu, Janeesh, MD at 4/22/2025 10:20 AM

Medical Oncology Clinic Note

Patient Name: Gary Lee Carrigan

MRN: 001256478

Date of Birth: 6/19/1950 74 y.o.

Referring Physician:No referring provider defined for this encounter.

Encounter Date: 4/22/2025

Chief Complaint:

Chief Complaint

Patient presents with

- Follow-up

Interval history

Patient returns to clinic after last appointment, bronchoscopy. He fell few days ago and hurt his right shoulder and his left hand. His right hand is feeling numb and he is going to see his primary care physician for that. Otherwise no new complaints. No worsening shortness of breath, cough, hemoptysis, fevers or chills.

Current Treatment Regimen:

Carboplatin and Taxol with or without radiation

Oncology History:

Patient with long standing history of pulmonary fibrosis. Incidental finding of right lung nodule in the fall of 2022.

Repeat CT scan in February 2023 showed enlarging paratracheal and mediastinal lymphadenopathy. Status post bronchoscopy with biopsy-non-small-cell lung cancer (squamous cell carcinoma) involvement of level 4, 7 and 10 lymph nodes. PDL1 70%.

PET/CT notable for 1.1 cm hypermetabolic right upper lobe nodule with diffuse adenopathy and a T6 lesion

–CT head without evidence of metastatic disease

–PD-L1 noted to be 70% on initial testing. No enough tissue for molecular testing

–Guardant360 testing without any actionable mutation

- started on pembrolizumab single agent.

–CT C/A/P in July 2023 showing overall treatment response

–CT C/A/P in October 2023 continued treatment response

–CT C/A/P in January 2024 without evidence of recurrent or metastatic disease

–CT C/A/P in April 2024 without evidence of recurrent or metastatic disease. Stable 6 mm right upper lobe nodule noted

–CT C/A/P in July 2024 without evidence of recurrence of disease. Stable 6 mm right upper lobe nodule noted

–CT C/A/P in December 2024 without evidence of significant disease progression or recurrence

–CT C/A/P in March 2025 reviewed and showing disease progression within multiple lymph node conglomerates in his lungs

CT Chest Abdomen Pelvis With Contrast 3/18/2025

Findings: Chest: There is a background of lower lung predominant pulmonary interstitial fibrosis with end-stage honeycombing. Mild scarring in the lung apices. Stable 8 mm confluent density in the posterior right lower lobe image #65 of series 2. There is cylindrical bronchiectasis in the lower lobes with peribronchial thickening. Partial opacification of the left lower lobe bronchi likely due to mucous plugging appears new as compared to prior. Central airways are otherwise patent. No pleural fluid identified. Confluent lymphadenopathy in the precarinal region has increased in size, measuring 3.4 x 2.2 cm (2.5 x 1.6 cm when measured similarly). Subcarinal confluent lymphadenopathy has also increased in size, now measuring 6 cm x 3.6 cm (3.8 x 2.6 cm when measured similarly). Additional prominent lymph nodes in the right hilum and right paratracheal region also appear increased. No axillary or supraclavicular adenopathy identified. Right subclavian cardiac stimulator device noted. Visualized thyroid is unremarkable. Coronary and other vascular calcification is noted. No aggressive osseous lesions are identified. Abdomen and pelvis: The spleen, pancreas, adrenals, liver are grossly unremarkable. Gallbladder is contracted but otherwise unremarkable. Bilateral probable renal cysts. Punctate 2 mm nonobstructing calculus in the left renal collecting system. Urinary bladder is unremarkable. Prostate is enlarged. No evidence of bowel obstruction, free air or free fluid. The appendix is normal. Moderate to large colonic fecal loading is noted. No gross adenopathy is

identified in the abdomen or pelvis. There is normal opacification of the mesenteric vessels. Aortic and other vascular calcification is noted. There is degenerative change in the spine. L3-L5 posterior fusion hardware appears unchanged.

Impression: Impression: 1.Interval further increase in size of mediastinal lymphadenopathy, suggesting progression of disease within the thorax. 2.No evidence of metastatic disease within the abdomen or pelvis.

Baptist oncologist Dr. Peterson discussed carboplatin, single agent Taxotere, combination of Taxotere and ramucirumab.

Patient is here for a 2nd opinion.

PATH

Final Diagnosis

LYMPH NODE, STATION 7, TRANSBRONCHIAL NEEDLE ASPIRATION (OUTSIDE CASE, COLLECTED 3/8/23):

- RARE MALIGNANT CELLS COMPATIBLE WITH METASTATIC NON-SMALL CELL CARCINOMA.

LYMPH NODE, STATION 4R, TRANSBRONCHIAL NEEDLE ASPIRATION (OUTSIDE CASE, COLLECTED 3/8/23):

- METASTATIC SQUAMOUS CELL CARCINOMA (SEE COMMENT).

LYMPH NODE, STATION 10R, TRANSBRONCHIAL NEEDLE ASPIRATION (OUTSIDE CASE, COLLECTED 3/8/23):

- RARE MALIGNANT CELLS COMPATIBLE WITH METASTATIC NON-SMALL CELL CARCINOMA.

Fine needle
aspiration - Core
Biopsy: C25-04482
4/15/25

Component

Final Diagnosis

A. LYMPH NODE, STATION 7, ENDOBRONCHIAL ULTRASOUND GUIDED FINE NEEDLE ASPIRATION:

- POSITIVE FOR MALIGNANCY, METASTATIC NON-SMALL CELL CARCINOMA (SEE COMMENT).

B. LYMPH NODE, 4 RIGHT, ENDOBRONCHIAL ULTRASOUND GUIDED FINE NEEDLE ASPIRATION:

- POSITIVE FOR MALIGNANCY, METASTATIC NON-SMALL CELL CARCINOMA (SEE COMMENT).

Fine needle aspiration - Core Biopsy: C25-04482

Order: 210812509 ☹️

Collected 4/15/2025 12:47

Fine needle
aspiration - Core
Biopsy: C25-04482
4/15/25

Status: Final result
Test Result Released: Yes (seen)
0 Result Notes

Component
Final Diagnosis
A. LYMPH NODE, STATION 7, ENDOBRONCHIAL ULTRASOUND GUIDED FINE NEEDLE ASPIRATION: - POSITIVE FOR MALIGNANCY, METASTATIC NON-SMALL CELL CARCINOMA (SEE COMMENT).
B. LYMPH NODE, 4 RIGHT, ENDOBRONCHIAL ULTRASOUND GUIDED FINE NEEDLE ASPIRATION: - POSITIVE FOR MALIGNANCY, METASTATIC NON-SMALL CELL CARCINOMA (SEE COMMENT).

4.8.25- 1st medical oncology appointment University of Kentucky. Reported chronic shortness of breath, stable chronic cough, anorexia that is getting better with Remeron. Discussed scans and recommended biopsy-1. For ruling out transformation of small-cell carcinoma, 2. Tissue for next generation sequencing

Past Medical, Surgical, Family and Social History:

- Medical History^[1]
- Surgical History^[2]
- Family History^[3]
- Social History^[4]

Reviewed past medical history, surgical, social family history. He quit smoking in 2020. Longstanding history of pulmonary fibrosis. He currently is a driver for Lexington school system but is a sergeant with Lexington police department.

Allergies:

Fosinopril

Medications:

- Current Medications^[5]
- Reviewed

Review of Systems:

A comprehensive 14 point review of systems was performed, noted to be negative with the pertinent positives documented in the HPI section of this note.

Vital Signs:

Visit Vitals

BP	130/74 (BP Location: Left arm)
Pulse	81

Temp36.3 °C (97.3 °F) (Oral)

Resp18

Physical Examination:
GENERAL: no acute distress.
SKIN: Left hand dorsum scabs
EYES: conjunctiva clear,
ENT: intact, no apparent injury
HEAD/NECK: neck supple
RESPIRATORY: non-labored respirations
CARDIOVASCULAR: regular rate and rhythm, no peripheral edema
GASTROINTESTINAL: soft, non-tender, non-distended,
MUSCULOSKELETAL: **No significant muscle wasting noted**
NEUROLOGICAL: alert and oriented x3, no focal deficits
PSYCHOLOGICAL: Normal mood.
EXT- left hand dorsum with some scabs.

Labs, Imaging, Pathology:

Lab Results

Component	Value	Date
WBC	8.85	04/22/2025
RBC	4.51 (L)	04/22/2025
HGB	11.8 (L)	04/22/2025
HCT	36.7 (L)	04/22/2025
MCV	81	04/22/2025
MCHC	32.2	04/22/2025
RDW	16.4 (H)	04/22/2025
PLT	334	04/22/2025
MPV	8.8	04/22/2025

Lab Results

Component	Value	Date
BUN	17	04/22/2025
CL	101	04/22/2025
NA	138	04/22/2025
K	5.0 (H)	04/22/2025
TP	7.2	04/22/2025
AST	29	04/22/2025
ALT	19	04/22/2025

Blood pressure 130/74, pulse 81, temperature 36.3 °C (97.3 °F), temperature source Oral, resp. rate 18, height 1.676 m (5' 5.98"), weight 63.9 kg (140 lb 14 oz), SpO2 97%.

I visualized the recent imaging and discussed the current radiology findings with the patient in detail and answered all questions.
=== 03/14/25 ===

CT THORACIC OUTSIDE IMAGES

No results found for this or any previous visit.

Performance Status:

ECOG 1

Assessment and Plan:

Gary Lee Carrigan is a 74 y.o. male with PMH of pulmonary fibrosis, former smoker with metastatic squamous cell carcinoma of the lung status post single agent pembrolizumab from July 20, 2023 to February 20, 2025. Now with aggressive disease progression mostly within the chest.

Metastatic squamous cell carcinoma of the lung

Cancer management : Gary Lee Carrigan is diagnosed with pulmonary fibrosis and metastatic squamous cell carcinoma of the lung.

This represents a life threatening illness for which urgent cancer treatment is indicated.

-bronchoscopy in 2023 with biopsy-non-small-cell lung cancer (squamous cell carcinoma) involvement of level 4, 7 and 10 lymph nodes. PDL1 70%.

PET/CT notable for 1.1 cm hypermetabolic right upper lobe nodule with diffuse adenopathy and a T6 lesion

– started on pembrolizumab single agent and with great response from July 2023 to March 2025..

- CT abdomen in March of 2025 shows confluent lymphadenopathy in the precarinal region, increased in size 3.4 x 2.2 cm. Subcarinal lymphadenopathy has also increased in size measuring 6 cm x 3.6 cm.

Also increased right hilar and right paratracheal region lymph nodes.

- 4/15 with biopsy of the lymph node station 7, 4R that showed metastatic non-small-cell lung cancer.

- Treatment goal- palliative

- Next Gen Sequencing: PD-L1 noted to be 70% on initial testing. No enough tissue for molecular testing, Guardant360 testing without any actionable mutation

- - this appears to be hyper progression within the chest. Abdomen with no evidence of disease. My concern was if this is small-cell transformation.

- the only site of metastatic disease is in the T6 as shown in a PET scan in March of 2023.

PLAN

His cancer is limited to within the thorax and so I believe he might benefit from chemoradiation concurrently. Plan to discuss in tumor board.

Referral sent to thoracic surgery.

Consent obtained for carbotaxol-if with concurrent chemoradiation I plan to do weekly carbotaxol, if only chemotherapy I plan to do every 3 weeks of carbotaxol.

Caris is pending for molecular sequencing.

At this point no clinical trials available.

- RTC 2 weeks

I updated the plan of care. This treatment will require ongoing monitoring of toxicity by me, due to the risks of severe side effects from the therapy listed above. The selection, dosing and administration of anti-cancer agents and the management of associated toxicities requires complex medical decision making. Modifications of drug dose and schedule as well as the initiation of supportive care interventions are often necessary because of expected toxicities. This varies individually based on patient tolerability, prior treatments and comorbidities. The optimal delivery of anticancer agents requires a healthcare delivery team experienced in the use of anticancer agents and the management of associated toxicities in patients with cancer.

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University of Kentucky.

[1]

Past Medical History:

Diagnosis	Date
- Allergies - Arthritis - Cancer (CMS/HCC) - CHF (congestive heart failure) (CMS/HCC) - Coronary artery disease - Ear problem - GERD (gastroesophageal reflux disease) - Heart problem - High cholesterol - Lung cancer (CMS/HCC) - Lung trouble - Myocardial infarction (CMS/HCC) - Sinus trouble - Sleep apnea - Sleep trouble	06/97 06/97 8/23 06/97 10/2006

[2]

Past Surgical History:

Procedure	Laterality	Date
- APPENDECTOMY - BACK SURGERY - COLONOSCOPY		1960 2019 ????

[3]

Family History

Problem	Relation	Name	Age of Onset
- Lung cancer - Lung cancer	Mother Father		

- Bone cancer Brother Velmo Jones
- Heart attack Brother Velmo Jones 60 - 69

[\[4\]](#)

Social History

Tobacco Use

- Smoking status: Former
 - Current packs/day: 0.00
 - Average packs/day: 1.5 packs/day for 50.4 years (75.6 ttl pk-yrs)
 - Types: Cigarettes
 - Start date: 1/12/1970
 - Quit date: 6/18/2020
 - Years since quitting: 4.8
- Smokeless tobacco: Never

Vaping Use

- Vaping status: Never Used

Substance Use Topics

- Alcohol use: Never
- Drug use: Never

[\[5\]](#)

Current Outpatient Medications:

- albuterol 108 (90 Base) MCG/ACT inhaler, Inhale 2 puffs., Disp: , Rfl:
- aspirin 81 MG EC tablet, Take 1 tablet (81 mg) by mouth 1 (one) time each day., Disp: , Rfl:
- cholecalciferol (Vitamin D-3) 25 MCG (1000 UT) tablet, Take 1 tablet (1,000 Units) by mouth 1 (one) time each day., Disp: , Rfl:
- Coenzyme Q10 (CO Q 10 PO), Take by mouth., Disp: , Rfl:
- diclofenac (Voltaren) 1 % topical gel, Disp: , Rfl:
- ezetimibe (Zetia) 10 MG tablet, Disp: , Rfl:
- hydrocortisone 2.5 % cream, Apply 1 Application topically., Disp: , Rfl:
- hydrOXYzine HCl (Atarax) 25 MG tablet, Take 1 tablet by mouth., Disp: , Rfl:
- loratadine (Claritin) 10 MG tablet, Take 1 tablet by mouth daily., Disp: , Rfl:
- Magnesium 200 MG tablet, Disp: , Rfl:
- metoclopramide (Reglan) 5 MG tablet, Take 1 tablet by mouth in the morning and 1 tablet at noon and 1 tablet in the evening and 1 tablet before bedtime., Disp: , Rfl:
- mirtazapine (Remeron) 15 MG tablet, Take 1 tablet by mouth 1 (one) time each day., Disp: , Rfl:
- Ofev 150 MG capsule, Disp: , Rfl:
- ondansetron ODT (Zofran-ODT) 8 MG disintegrating tablet, Place 1 tablet (8 mg) under the tongue., Disp: , Rfl:
- pantoprazole (Protonix) 40 MG EC tablet, Take 1 tablet (40 mg) by mouth 1 (one) time each day., Disp: , Rfl:
- rosuvastatin (Crestor) 20 MG tablet, Disp: , Rfl:
- tamsulosin (Flomax) 0.4 MG 24 hr capsule, Take 1 capsule by mouth 1 (one) time each day., Disp: , Rfl:
- TURMERIC PO, Take 1,000 mg by mouth., Disp: , Rfl:
- Zinc Sulfate 66 MG tablet, Disp: , Rfl:
- azithromycin (Zithromax) 250 MG tablet, Disp: , Rfl:

- doxycycline (Vibramycin) 100 MG capsule, , Disp: , Rfl:
- Fluticasone-Umeclidin-Vilant 100-62.5-25 MCG/ACT aerosol powder , Inhale. (Patient not taking: Reported on 4/22/2025), Disp: , Rfl:
- HYDROcodone-acetaminophen (Norco) 5-325 MG tablet, Take 1 tablet (5 mg of hydrocodone) by mouth. (Patient not taking: Reported on 4/22/2025), Disp: , Rfl:
- irbesartan-hydroCHLOROthiazide (Avalide) 300-12.5 MG tablet, Take 1 tablet by mouth 1 (one) time each day in the morning. (Patient not taking: Reported on 4/8/2025), Disp: , Rfl:
- mirtazapine (Remeron) 15 MG tablet, Take 1 tablet by mouth nightly., Disp: , Rfl:
- Nintedanib Esylate 150 MG capsule, Take by mouth., Disp: , Rfl:
- pembrolizumab (Keytruda) 100 MG/4ML injection, Infuse into a venous catheter. (Patient not taking: Reported on 4/8/2025), Disp: , Rfl:
- predniSONE (Deltasone) 20 MG tablet, , Disp: , Rfl: